INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Roswell Park modified fluorouracil + folinic acid

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
56 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00427
Version 6

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

Treatment stages

01 **Prescribed treatment course**
Adjuvant or metastatic colorectal cancer

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Roswell Park modified fluorouracil + folinic acid
National Cancer Control Programme (NCCP) 00427

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make deoxyribonucleic acid (the cell's genetic material) (DNA). In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

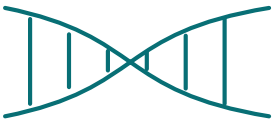
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

Your course

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Understanding the evidence

Ask which evidence matches your treatment aim.

NCCP cites NSABP C-03, which studied postoperative fluorouracil/leucovorin in colon cancer. This historical evidence does not establish an individual benefit for every metastatic setting or every modified folinic-acid schedule.

CHAPTER 02

Your schedule, tests and appointments

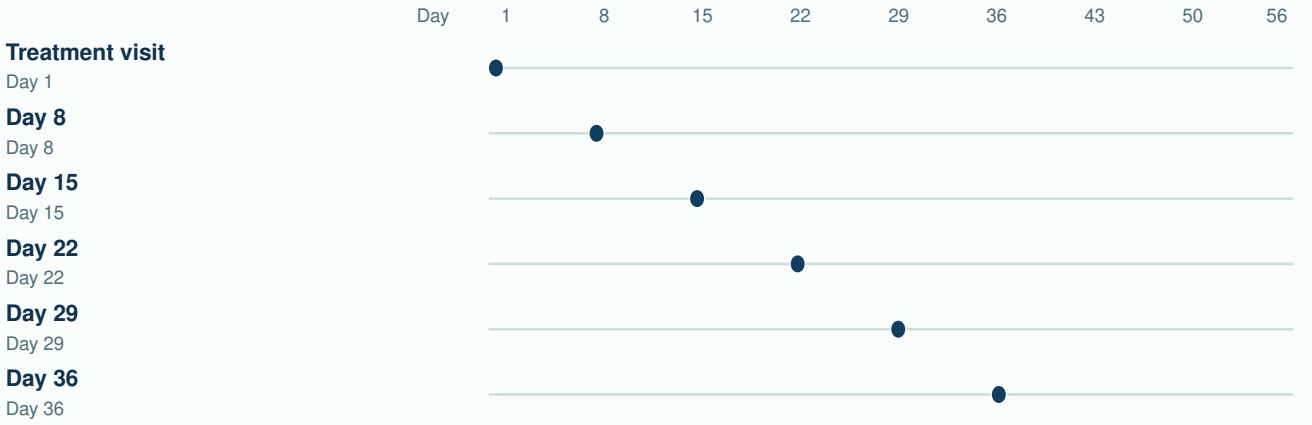
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment plan

Roswell Park modified fluorouracil + folinic acid

56-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ADJUVANT OR METASTATIC COLORECTAL CANCER

Prescribed treatment course

Start | **Treatment** | **Review**

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00427 v6 official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Roswell Park modified fluorouracil + folinic acid: treatment and checks

Adjuvant or metastatic colorectal cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00427	Baseline tests and DPD status	Before every weekly treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00427	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00427	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up

What each check is for

1 Confirm your personal plan

Before starting

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

Visits and access

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.This regimen does not require a home infusion pump.

STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Blood tests

Before every weekly treatment: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit	 **Day 1** Fluorouracil + folinic acid
Day 8	 **Day 8** Fluorouracil + folinic acid
Day 15	 **Day 15** Fluorouracil + folinic acid
Day 22	 **Day 22** Fluorouracil + folinic acid
Day 29	 **Day 29** Fluorouracil + folinic acid
Day 36	 **Day 36** Fluorouracil + folinic acid
Planned rest	 **Days 43–56** No catch-up doses; symptoms may persist

The written plan comes first: Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00427 v6 official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1–42: Six weekly treatment visits

Attend on Days 1, 8, 15, 22, 29 and 36; both medicines at each visit. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Day 43–56: Planned two-week break

No catch-up doses; symptoms can continue during the break. Confirm the next plan. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Support around the medicines

1 **Supportive medicines**

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 **Mouth, bowel and skin care**

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 **Medicine checks**

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Energy / daily life

Report fatigue that limits usual activity; ask for practical and nutrition support.

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Questions worth asking

Which treatment plan applies to me?

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Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fluorouracil treatment day

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

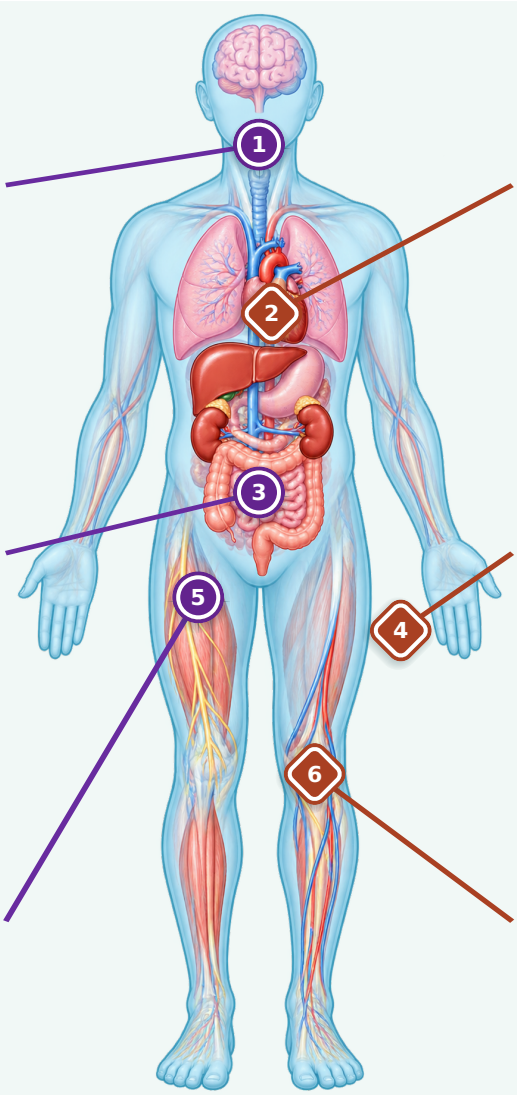
Fluorouracil

Report painful ulcers or difficulty eating and drinking.

2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.



3 Bowel / hydration

Fluorouracil

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

6 Energy / daily life

Treatment

Report fatigue that limits usual activity; ask for practical and nutrition support.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00427 v6 official regimen**
   https://healthservice.hse.ie/documents/6466/427_Roswell_Park_Modified.pdf
2. **NCCP NCIS antiemetic guidance V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **NSABP C-03 primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/8410113/
4. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq